

STEADY

Between-Visit Care Command Center

Implementation-Ready Engineering Handoff - Updated Command Center UX Specification

Clinical Intelligence Expansion | Document 03 | Repository-grounded against tprawlins-lang/Emdr | branch claude/gifted-keller-501y5d | September 2026

Build directive. Turn Steady clinician home into one clinical attention surface. The Command Center answers: **Who needs me today, why, what changed, and what should I do next?** It combines existing safety and operational work with Return-to-Life, Response Fingerprint, Recovery Trajectory, Therapeutic Load, clinician follow-ups, Session Prep, check-ins, assessments, authorized Companion evidence, and patient activity without becoming an alert wall.

Product rule. Steady may determine what information deserves attention according to deterministic, versioned policy. It does not decide clinical meaning for the clinician. Model output may compress evidence into readable language, but it cannot set urgency, ownership, due date, safety state, treatment intensity, or queue order.

Repository foundation this handoff assumes

Foundation	Current Steady anchor	Rule for this build
Longitudinal history	src/lib/spine.ts + ADR 0010	Durable state is event-backed and replayable. No parallel mutable history.
Tenant/person boundary	src/lib/repository.ts + ADR 0011	Every new patient-bound record carries tenant_id and person_id. New paths use TenantContext.
Clinician workspace	src/app/clinician/member/[id]/*	Extend PersonShell, evidence drill-down, and clinician review conventions.
Evidence summary	src/lib/clinical/summary.ts	A material generated claim without resolvable evidence is withheld.
Trajectory	src/lib/clinical/trajectory.ts	Separate scales remain separate; provenance visible; no arbitrary composite score.
Work queue	src/lib/clinical/work-queue.ts	Clinician home opens on work; server ordering; plain-language reason; one primary action.
Companion memory	src/lib/companion.ts	Patient/Companion evidence keeps source class and visibility rules. Companion output is not clinician evidence.
Clinician Thoughts / Session Prep	Engineering Spec v2.1	Use approved clinician memory, follow-ups, full longitudinal prep, and provenance.
AI Gateway	ADR 0012	New model tasks use schema validation, task/model versioning, provenance, PHI policy, and evaluation.

Cross-feature invariants

- **Safety authority stays deterministic.** Command Center displays safety work but cannot clear, weaken, bypass, or replace the safety engine.
- **Source classes never collapse.** Patient report, clinician observation, formal note, system measurement, Companion interaction, and model inference remain distinguishable.
- **Associations remain associations.** Temporal overlap or statistical pattern is not causation.
- **No future-data leakage.** Historical views use only evidence available at the selected cutoff.
- **Missing data stays missing.** Missing does not become zero, normal, stable, compliant, or recovered.
- **Corrections append.** Corrected derived state supersedes prior state without erasing history.
- **No alert inflation.** Review-worthy intelligence does not become a safety alert.
- **Stable is a valid outcome.** The system must be able to say no clinician action is suggested.

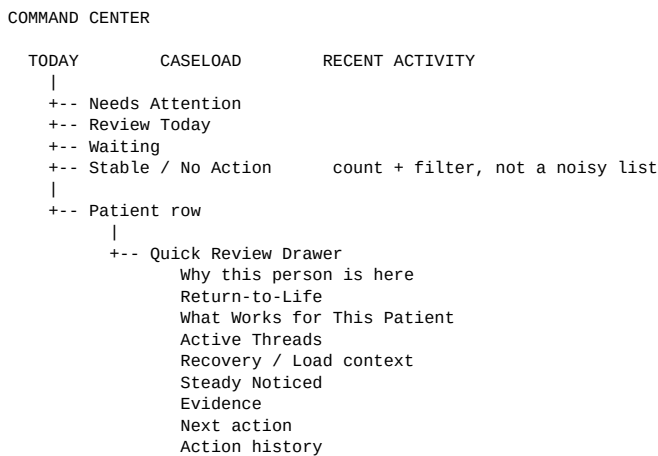
1. Product outcome and visual philosophy

The Command Center is a clinical cockpit, not a reporting dashboard. The default surface shows work, not charts. Population analytics, billing, revenue, organization KPIs, and detailed measure charts belong elsewhere. The first screen is intentionally narrow: identify attention, explain it, and make the next action easy.

The first 10 seconds

- How many people need attention now?
- How many should be reviewed today?
- What is waiting on somebody else?
- How much of the caseload is currently stable with no suggested action?
- For each person shown, why are they here and what is the one best next action?

2. Top-level information architecture



The underlying work queue may retain more granular machine groups. The UI maps those groups into four clinician-readable buckets. Recently resolved work belongs in Recent Activity and patient history, not as a permanent front-page section.

UI group mapping

UI group	Underlying sources	Display rule
Needs Attention	Existing immediate/high safety obligations; overdue high-priority clinician obligations; policy-approved review_now signals	Safety remains visibly labeled as safety. Non-safety review_now cannot masquerade as safety.
Review Today	review_today signals; due clinician follow-ups; meaningful new longitudinal change	Non-urgent. Explain why review is useful today.
Waiting	waiting_member; waiting_staff; known referral/integration/data dependency	State the dependency. Patient silence is not noncompliance.
Stable / No Action	Active authorized caseload with no open actionable work under current policy	Derived count/filter, not a durable signal. Clicking opens filtered Caseload.

3. TODAY screen - exact product contract

STEADY COMMAND CENTER				Thu Sep 3	
3	Need Attention	6	Review Today	4	Waiting 18 Stable
NEEDS ATTENTION					
Sarah M.				HIGH	
Recovery trajectory changed 6 days ago					
Sleep worsening Return-to-Life goal stalled 3 difficult check-ins					
Steady noticed					
Grounding still appears helpful, but recovery after sessions is taking longer than her recent baseline.					
Last clinician contact: 8 days ago				[Review Sarah]	
Michael R.				REVIEW TODAY	
Follow-up due: ask about returning to work					
Return-to-Life: 3 of 5 steps reached					
Symptoms improving No active safety obligation					
[Open Session Prep]					
David L.				REVIEW TODAY	
Progress has flattened across the current review window					
Exposure response mixed Sleep little changed					
[Review Trajectory]					

Header strip

- Counts are clickable filters: Need Attention, Review Today, Waiting, Stable.
- Counts are computed for the clinician's authorized caseload and active tenant.
- Stable count is not a quality score. It means no current work item under the active policy.
- Show projection freshness and policy version in a low-emphasis status affordance.

4. Patient row anatomy

Field	Required behavior
Identity	Patient display name plus minimal identity context already approved for clinician surface.
Attention state	Needs Attention / Review Today / Waiting. Safety obligation uses existing safety wording and treatment.
Primary reason	One plain-language sentence naming why the row exists. Never an unexplained score.
Supporting facts	Maximum three short facts. Each maps to evidence IDs and preserves source class.
Cross-system sentence	Optional one-sentence synthesis across approved systems. It cannot create the row, change band, or choose action.
Change	Use new since last review, worsened since, goal unchanged only when deterministic comparison supports it.
Owner / last contact	Named current owner when available and last meaningful clinician contact.
One primary action	Exactly one row-level action from deterministic workflow rules. Secondary actions live in the drawer.

Row density rule. A row is not a mini chart. The clinician understands the reason without opening it; scores, history, full evidence, and extra actions belong in the drawer.

5. Quick Review Drawer - exact sections

+-----+-----+	
Sarah M.	[x]
Last session: Aug 28	Next appointment: Sep 5
+-----+-----+	
WHY SHE'S HERE	
Recovery trajectory changed	
Sleep: 6 -> 3 over 9 days	
Functional goal: no progress for 17 days	
Post-session recovery: taking longer	
[View evidence]	
+-----+-----+	
RETURN TO LIFE	
Grocery shopping	
Level 3 of 5 No new milestone since Aug 17	
+-----+-----+	
WHAT WORKS FOR THIS PATIENT	
Grounding Usually helpful	
BLS Helpful in-session; slower delayed recovery	
Journaling Mixed observed response	
+-----+-----+	
ACTIVE THREADS	
Abandonment / separation	
Sleep disruption	
Sister relationship	
+-----+-----+	
RECOVERY / LOAD CONTEXT	
Current trajectory: slowing	
Therapeutic load: maintain / review evidence	
+-----+-----+	
STEADY NOTICED	
Sleep worsening overlaps with increased family-related	
distress in the current evidence window.	
Association only 5 supporting sources	
[Why am I seeing this?]	
+-----+-----+	
NEXT ACTION	
[Open Session Prep]	
Other: Record Thought Message Add Follow-up Mark Reviewed	
+-----+-----+	

Drawer rules

- **Why this person is here** comes from the durable work item and deterministic provider.
- **Return-to-Life** appears only when there is an active goal and clinician access.
- **What Works for This Patient** uses Response Fingerprint observations. Wording is observed association, never treatment truth.
- **Active Threads** uses accepted clinician threads and approved memory. Proposed AI links do not appear as accepted.
- **Recovery / Load Context** appears after Handoffs 04 and 05 and never duplicates the safety engine.
- **Steady Noticed** is visually distinct and explicitly model-derived. Every material claim is evidence-linked.
- The drawer may expose secondary actions, but the queue row retains exactly one primary action.
- Opening the drawer is not acknowledgement. Acknowledgement is explicit.

6. CASELOAD view

Caseload answers a different question from Today: **What is the current shape of my whole caseload?** It is still clinical, not operational analytics.

Patient	Function	Trajectory	Response	Load/Readiness	Last contact
Sarah	Stalled	Slowing	Mixed	Maintain/review	8d
Michael	Improving	Improving	Supportive	No review needed	3d
David	Stalled	Little change	Mixed	Review	5d
Lauren	Improving	Improving	Supportive	No review needed	2d

Caseload column rules

- Function comes from Return-to-Life and authorized functional evidence. No goal means Not set, not zero.
- Trajectory comes from Handoff 04 and uses descriptive states, not a universal recovery score.
- Response comes from Handoff 02 and must show supportive/mixed/insufficient evidence honestly.
- Load/Readiness comes from Handoff 05 and remains clinician decision support, not a treatment order.

- Every label opens evidence, calculation window, limitations, and source dates.
- Default sort is current clinical need. User filters do not rewrite server clinical priority semantics.

7. RECENT ACTIVITY view

Recent Activity is a clinician situational-awareness feed built from authorized events. It is not a raw audit log and not a firehose of every patient action.

```

9:18 AM Sarah completed check-in
Sleep quality decreased to 3

8:42 AM Michael reached Return-to-Life milestone
Drove to work independently

Yesterday David completed grounding practice
Distress 7 -> 4 during the recorded exercise

Yesterday Sarah had an authorized Companion interaction
Family conflict discussed
No new safety obligation created

```

Activity inclusion rules

- Include clinically relevant check-ins, measures, sessions, Return-to-Life milestones, intervention response, clinician thought save, relevant Companion interaction metadata, safety obligations, and follow-up state changes.
- Do not show raw Companion transcript text in the clinician-wide feed.
- Companion entries obey source visibility and consent. Metadata cannot bypass a content visibility restriction.
- Collapse repetitive same-type events when individual items add no clinical meaning.
- Every feed item links to patient and evidence when access permits.
- Default chronological order. Optional clinically relevant filters remain server-defined and explainable.

8. Cross-system attention synthesis

The Command Center may show one concise sentence joining information across systems. It sits on top of deterministic work generation; it is not a hidden prioritization model.

Example: Function stalled, recovery slowed, grounding still appears helpful, session recovery is taking longer, and you planned to revisit sleep.

```

interface ClinicianAttentionSummaryV1 {
  schemaVersion: "1";
  personId: string;
  signalId: string;
  text: string;
  evidenceIds: string[];
  sourceClasses: Array<
    "clinician_documented" |
    "clinician_observed" |
    "patient_reported" |
    "system_measured" |
    "model_derived"
  >;
  taskVersion: string;
  generatedAt: string;
}

Hard validation:
- signal exists before generation
- every material clause has evidence
- evidence belongs to same tenant/person
- model cannot alter attention band
- model cannot set owner, due date, action, safety state
- unsupported clauses are dropped
- entire sentence may be withheld if remainder becomes misleading

```

9. Durable attention signal model

Do not overload the alerts table with every non-safety intelligence output. Safety alert semantics must stay believable. Create a separate attention-signal model that the existing work queue merges with safety and caseload work.

```
CREATE TABLE clinical_attention_signals (  
  id TEXT PRIMARY KEY,  
  tenant_id TEXT NOT NULL,  
  person_id TEXT NOT NULL,  
  signal_type TEXT NOT NULL,  
  source_feature TEXT NOT NULL,  
  dedupe_key TEXT NOT NULL,  
  attention_band TEXT NOT NULL CHECK (attention_band IN (  
    'review_now', 'review_today', 'follow_up', 'watch'  
  )),  
  statement TEXT NOT NULL,  
  change_text TEXT,  
  state TEXT NOT NULL CHECK (state IN (  
    'open', 'acknowledged', 'waiting_member', 'waiting_staff',  
    'resolved', 'dismissed'  
  )),  
  owner_person_id TEXT,  
  due_at TEXT,  
  first_detected_at TEXT NOT NULL,  
  last_detected_at TEXT NOT NULL,  
  evidence_at TEXT NOT NULL,  
  policy_version TEXT NOT NULL,  
  limitations_json TEXT NOT NULL DEFAULT '[]',  
  created_at TEXT NOT NULL,  
  updated_at TEXT NOT NULL,  
  UNIQUE(tenant_id, person_id, dedupe_key)  
);  
  
CREATE TABLE clinical_attention_signal_evidence (  
  signal_id TEXT NOT NULL,  
  evidence_type TEXT NOT NULL,  
  evidence_id TEXT NOT NULL,  
  rank INTEGER NOT NULL,  
  PRIMARY KEY(signal_id, evidence_type, evidence_id)  
);  
  
CREATE TABLE between_visit_care_actions (  
  id TEXT PRIMARY KEY,  
  tenant_id TEXT NOT NULL,  
  person_id TEXT NOT NULL,  
  clinician_person_id TEXT NOT NULL,  
  signal_id TEXT,  
  action_type TEXT NOT NULL,  
  note TEXT,  
  started_at TEXT,  
  completed_at TEXT NOT NULL,  
  duration_seconds INTEGER,  
  outcome_state TEXT,  
  source_surface TEXT NOT NULL,  
  created_at TEXT NOT NULL  
);
```

Required indexes

- clinical_attention_signals(tenant_id, owner_person_id, state, attention_band, due_at)
- clinical_attention_signals(tenant_id, person_id, state, last_detected_at)
- clinical_attention_signal_evidence(signal_id, rank)
- between_visit_care_actions(tenant_id, person_id, completed_at)
- between_visit_care_actions(tenant_id, clinician_person_id, completed_at)

Stable is not stored as a signal

Stable / No Action is a projection outcome: an authorized active caseload member with no open safety obligation, no open attention signal, and no due clinician obligation under current policy. Do not insert thousands of stable rows into the signal table.

10. Signal provider contract

```
interface AttentionSignalProvider {
  id: string;
  version: string;
  evaluate(args: {
    tenantId: string;
    personId: string;
    evidenceCutoff: string;
  }): Promise<AttentionSignalCandidate[]>;
}

interface AttentionSignalCandidate {
  type: string;
  dedupeKey: string;
  band: "review_now" | "review_today" | "follow_up" | "watch";
  statement: string;
  changeText?: string | null;
  evidenceIds: string[];
  evidenceAt: string;
  limitations: string[];
  policyVersion: string;
}
```

Provider registry after this series

Provider	Purpose
existing-alert-adapter	Expose existing safety/operational work without changing its semantics.
explicit-followup-provider	Clinician-authored obligations and follow-up dates.
return-to-life-provider	Meaningful goal evidence, stall/review windows, milestone changes.
response-fingerprint-provider	Repeated supported/mixed/delayed response patterns that deserve review.
engagement-gap-provider	Observed gap only. Never predicted disengagement.
recovery-trajectory-provider	Handoff 04 trajectory stall/slowing/reversal signals.
therapeutic-load-provider	Handoff 05 clinician review signals; cannot clear safety or prescribe progression.

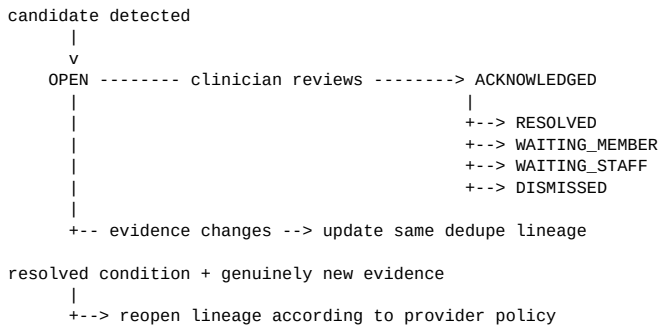
Signal generation boundary

- Providers are deterministic or rule-bound and versioned.
- Optional AI explanation runs only after a provider emits a candidate.
- A provider may emit review-worthiness, but only existing safety machinery may create safety authority.
- Later providers plug into the registry without changing work-queue semantics.

11. Ordering and one-action policy

- Existing immediate/high safety obligations remain first.
- Overdue explicit clinician obligations follow according to policy.
- Within non-safety work: policy-defined clinical importance, due state, evidence recency, stable ID tiebreak.
- Model output does not sort the queue.
- Duplicate same-person same-reason signals collapse and preserve evidence/event count.
- Exactly one primary row action is server-selected. Secondary actions appear only in the drawer.

12. Signal and review lifecycle



Lifecycle rules

- Opening a row or drawer does not silently acknowledge it.
- Acknowledgement records clinician, timestamp, evidence version/cutoff, and source surface.
- Dismiss requires a reason category and optional note.
- Waiting state names the dependency and may carry a follow-up date.
- Resolution never deletes underlying evidence.
- If a signal changes materially while open, update the row and expose new-since-review rather than creating duplicates.

13. Clinician actions and care-time ledger

Action	Behavior
review	Open evidence/Session Prep and explicitly record reviewed state.
contact	Launch existing contact workflow. Never send automatically.
add_followup	Create clinician follow-up linked to evidence and patient.
record_thought	Launch Record Thoughts inside correct patient context.
open_session_prep	Open full longitudinal Session Prep.
review_trajectory	Open patient trajectory with relevant window highlighted.
adjust_plan_link	Navigate to clinician-controlled plan surface. Signal cannot alter plan.
resolve	Close work item with reason/outcome.

- Capture between-visit clinician work time where practical for staffing, operations, and future reimbursement workflows.
- Do not mark time billable from Steady alone.
- Do not count passive browser-open time as clinical work.
- Clinician can correct or annotate recorded care time.

14. API / command surface

```
GET /api/clinician/command-center/today
GET /api/clinician/command-center/caseload
GET /api/clinician/command-center/activity

GET /api/clinician/member/:personId/attention/:signalId
POST /api/clinician/member/:personId/attention/:signalId/acknowledge
POST /api/clinician/member/:personId/attention/:signalId/state
POST /api/clinician/member/:personId/attention/:signalId/action

GET /api/clinician/member/:personId/command-context
```

Today response contract

```
{
  "computedAt": "...",
  "policyVersion": "...",
  "counts": {
    "needsAttention": 3,
    "reviewToday": 6,
    "waiting": 4,
    "stable": 18
  },
  "groups": {
    "needsAttention": [CommandCenterRow],
    "reviewToday": [CommandCenterRow],
    "waiting": [CommandCenterRow]
  }
}
```

CommandCenterRow minimum

```
{
  id, personId, personName, uiGroup, sourceKind,
  safetyAuthority, reason, supportFacts[0..3],
  crossSystemSummary?, change?, evidenceAt,
  lastClinicianContactAt?, owner?, dueAt?,
  primaryAction, actionable, blockedReason?, evidenceRefs[]
}
```

Tenant and clinician identity are resolved server-side. Browser-supplied `tenant_id` is ignored. Foreign-person IDs follow existing not-found semantics.

15. Command-context aggregation

The drawer should not issue independent raw queries from every component. Build a patient-scoped command-context service that assembles authorized summaries from each installed subsystem.

```
CommandContext
- activeWorkItem
- returnToLifeSummary?
- responseFingerprintSummary?
- activeThreads[]
- recoveryTrajectorySummary?
- therapeuticLoadSummary?
- sessionPreLink
- clinicianFollowups[]
- recentEvidence[]
- steadyNoticed?
- actionHistory[]
- coverage / missingData / withheldByPolicy
```

- Every subsystem adapter returns evidence refs and provenance.
- Missing subsystem data returns explicit unavailable/insufficient state.
- Command Center does not reach around subsystem rules to access protected content.
- Cache keys include tenant, person, evidence cutoff, policy versions, and relevant feature versions.

16. AI role and model boundaries

- No AI task is required to decide whether a work item exists.
- No AI task decides queue position, urgency, safety state, due date, owner, or next action.
- Optional AI task: clinician.command_center.summarize.
- Input is normalized, authorized evidence selected by deterministic services.
- Output is short, patient-scoped, evidence-linked, and validated before render.
- Patient/Companion text is untrusted content and cannot become instruction to the model.
- A summary that fails citation validation is withheld; deterministic reason remains.

Suggested AI task schema

```
CommandCenterSummaryV1
  schemaVersion: "1"
  signalId: string
  headline: string
  supportingFacts: [
    { text: string, evidenceIds: string[] }
  ] // max 3
  synthesis?: {
    text: string
    evidenceIds: string[]
    limitation: string
  }
```

Describe only what supplied evidence supports. Preserve source class, uncertainty, disagreement, and missing data. Do not diagnose, prescribe, infer causation, assign urgency, or recommend treatment intensity.

17. Events

Event	Meaning
attention_signal.opened.v1	New durable non-safety work item.
attention_signal.updated.v1	Same dedupe lineage changed due to new evidence.
attention_signal.acknowledged.v1	Clinician explicitly reviewed without resolving.
attention_signal.state_changed.v1	Waiting/resolved/dismissed lifecycle.
attention_signal.reopened.v1	Resolved condition recurred with new evidence.
between_visit_care.action_recorded.v1	Clinician action/time/outcome recorded.
command_center.summary_generated.v1	Optional generated presentation summary with task/evidence versions.

Durable clinical state changes use the longitudinal event envelope. Pure UI analytics are separate and contain no patient text or sensitive labels.

18. Security, privacy, and permissions

- All Today, Caseload, Activity, drawer, signal, and evidence calls require TenantContext and clinician authorization.
- Cross-patient and cross-tenant access fails before retrieval or model context assembly.
- Org administrators do not gain clinical content solely from administrative role.
- Recent Activity cannot surface raw Companion content in a clinician-wide feed.
- Evidence drill-down obeys source visibility; the Command Center cannot reveal restricted data by paraphrasing it.
- No patient text, Companion text, goal names, thread names, or clinical labels in external analytics.
- Audit acknowledgement, dismissal, state change, care action, and privileged evidence access where policy requires.
- Feature flags may disable downstream intelligence without deleting history.

19. Accessibility and interaction

- Status is not communicated by color alone.
- Keyboard navigation supports group filters, rows, drawer, evidence links, and actions.
- Drawer focus is trapped while open and returns to originating row on close.
- Primary action remains reachable without hover.
- Long reasons wrap; no essential text is tooltip-only.
- Screen-reader labels distinguish safety obligation, review signal, waiting dependency, and stable count.

20. Failure and empty states

State	Required behavior
No work today	Say No clinician action is suggested right now. Still show Caseload and Recent Activity navigation.
Projection stale	Show freshness warning and last successful compute time. Do not pretend queue is current.
One provider failed	Keep other work. Surface partial coverage without exposing PHI in telemetry.
AI summary failed	Render deterministic reason/support facts. Never hide the work item.
Evidence removed by policy	Keep row only if remaining authorized evidence supports it; otherwise update/withdraw per provider policy.
Patient access lost	Remove actionable controls and follow authorization behavior immediately.
Missing downstream feature	Show Not available or omit section. Never manufacture neutral state.

21. Implementation phases

Phase 1 - Signal substrate and UI mapping

- Add attention-signal schema, evidence links, indexes, provider interface.
- Extend work queue without changing safety ordering.
- Map deterministic state to Needs Attention / Review Today / Waiting.
- Compute Stable count from caseload minus open work.

Definition of done: Existing queue unchanged when feature is off; stable dedupe/order tested; safety retains current authority.

Phase 2 - TODAY screen

- Build header counts and filters.
- Build richer row with reason, max-three support facts, owner/contact, one primary action.
- Add freshness and partial-coverage state.

Definition of done: Clinician understands why each person appears without opening the drawer; no row relies on unexplained score.

Phase 3 - Quick Review Drawer and command-context service

- Build command-context aggregation and evidence adapters.
- Implement Why Here, Return-to-Life, Response Fingerprint, Active Threads, Steady Noticed, evidence, next actions, action history.
- Trajectory and Load sections remain feature-gated until Handoffs 04/05 land.

Definition of done: Every material statement can open its evidence; unavailable systems show honest missing state.

Phase 4 - Caseload and Recent Activity

- Build Caseload clinical state view.
- Build clinically relevant event feed with Companion visibility restrictions.
- Stable filter opens relevant caseload population.

Definition of done: Caseload has no composite score; activity is not a raw-event firehose.

Phase 5 - Optional cross-system synthesis

- Register AI Gateway task.
- Validate clause-to-evidence mapping.
- Keep deterministic row usable with AI disabled.

Definition of done: Model cannot alter row existence, group, owner, due date, action, or safety state; uncited synthesis withheld.

Phase 6 - Action ledger and downstream providers

- Complete care-action ledger and correction flow.
- Publish provider contract for Handoff 04 Recovery Trajectory and Handoff 05 Therapeutic Load.
- Add tenant-aware flags and policy registry.

Definition of done: Handoffs 04 and 05 plug in without changing queue semantics or data contracts.

22. Test matrix

Area	Required tests
Queue mapping	Safety/high -> Needs Attention; review signals -> Review Today; waiting states -> Waiting; no open work -> Stable count.
Ordering	Safety stays first; deterministic order stable across same evidence/policy; model has no effect.
Dedupe	Repeated same-person same-reason evidence updates one lineage; recurrence follows provider policy.
Tenant/person	Foreign tenant, foreign person, guessed signal ID, cross-patient evidence refs fail before retrieval.
Permissions	Unassigned/expired access; supervisor/coverage; nonclinical admin; blocked action reason.
Row summary	Max three facts; evidence-linked; unsupported generated clause withheld.
Drawer	Every section source-backed; missing features honest; active threads exclude unaccepted AI links.
Companion	Restricted interaction not exposed; patient statement remains patient-reported; model reply not evidence.
Caseload	No goal -> Not set; insufficient fingerprint -> Insufficient evidence; no composite score.
Activity	No raw Companion transcript; event collapse; chronological default; authorization enforced.
Care time	No passive time; explicit start/end/correction; action lineage preserved.
Point in time	Historical view does not use future evidence.
Replay	Signal/action durable state replayable where spine contract requires.
Failure	Provider failure gives partial coverage; AI failure falls back to deterministic content.

23. Acceptance criteria

- Clinician opens one Command Center, not separate alerts, AI alerts, and caseload lists.
- Today clearly shows Needs Attention, Review Today, Waiting, and Stable counts.
- Safety work remains visibly and semantically different from non-safety review signals.
- Each row says who, why, what changed, and one primary next action.
- Patient drawer joins Return-to-Life, Response Fingerprint, active clinician threads, later Recovery Trajectory, later Therapeutic Load, and Session Prep without flattening provenance.
- Steady Noticed is clearly model-derived and every material claim is evidence-linked.
- Caseload shows patient-specific clinical states rather than a universal health/recovery score.
- Recent Activity gives situational awareness without exposing restricted Companion content.
- Stable means no current suggested action under policy, not healthy or low risk.
- Signal generation, grouping, and ordering are deterministic and versioned.
- A dismissed/acknowledged/resolved signal has auditable human action.
- Handoffs 04 and 05 can add providers without changing queue contract.
- Feature works with AI disabled.

Appendix A. File-level implementation map

Path	Change
src/lib/clinical/attention-signals.ts	NEW signal lifecycle/repository and stable dedupe.
src/lib/clinical/attention-providers/*	NEW provider registry and adapters.
src/lib/clinical/work-queue.ts	EXTEND merge attention signals, UI grouping, stable count, preserve safety ordering.
src/lib/clinical/command-context.ts	NEW patient-scoped drawer aggregation and coverage state.
src/lib/clinical/command-summary.ts	NEW optional evidence-validated cross-system presentation summary.
src/lib/clinical/recent-activity.ts	NEW authorized clinically relevant activity projection.
src/app/clinician/today/page.tsx	UPDATE as Command Center Today default surface.
src/app/clinician/caseload/page.tsx	NEW/UPDATE caseload clinical state view.
src/app/clinician/activity/page.tsx	NEW recent activity view.
src/components/clinical/CommandCenterHeader.tsx	NEW counts, filters, freshness.
src/components/clinical/WorkQueueRow.tsx	EXTEND richer row while retaining one primary action.
src/components/clinical/AttentionSignalDrawer.tsx	NEW exact quick-review drawer sections.
src/components/clinical/CaseloadStateTable.tsx	NEW clinical state table with evidence drill-down.
src/components/clinical/RecentActivityFeed.tsx	NEW event feed with visibility-safe rendering.
src/lib/spine.ts + src/lib/projections.ts	ADD signal/action and optional summary events.
src/lib/db.ts + scripts/pg-schema.sql	ADD/mirror tables and indexes.
src/lib/ai-gateway/*	REGISTER optional clinician.command_center.summarize task.
tests/attention-signals*.test.ts	Dedupe, order, mapping, action lifecycle, replay.
tests/command-center*.test.ts	Today, drawer, caseload, activity, summary validation, failure states.
tests/tenant-isolation.test.ts	ADD signal/evidence/activity/command-context attack cases.

Appendix B. Feature flags

CLINICAL_COMMAND_CENTER
CLINICAL_ATTENTION_SIGNALS
CLINICAL_COMMAND_CENTER_DRAWER
CLINICAL_COMMAND_CENTER_CASELOAD
CLINICAL_COMMAND_CENTER_ACTIVITY
CLINICAL_COMMAND_CENTER_AI_SUMMARY

Flags should be tenant-aware when current configuration permits. Turning off presentation does not delete signal, action, or evidence history.

Appendix C. Final engineering instruction

Do not build the Command Center as a new dashboard beside the current clinician work queue. The Command Center is the evolution of that work queue. Preserve the existing who-needs-my-attention-and-why philosophy, server-side priority, source honesty, and one-row-action contract. Add the richer cross-system context here without turning Steady into an alert wall or black-box patient scoring system.